



OFFICIAL

# Communicating for Safety Procedure

## 1. Applicability

| Site                     | Operational Area   | Applicable to                                                                  |
|--------------------------|--------------------|--------------------------------------------------------------------------------|
| Armadale Kalamunda Group | All clinical areas | All clinical staff (medical, nursing, midwifery, allied health, support staff) |

## 2. Purpose

The purpose of this guideline is to support the principles outlined in the [Department of Health Western Australian \(HDWA\) Clinical Handover Policy](#) by providing practical clinical handover information to ensure timely, purpose-driven and effective communication and documentation which supports continuous, coordinated and safe care for patients across the patient journey.

## 3. General principles – communicating for safety

### 3.1 Patient/carer involvement

- In a patient-centred approach to care, it is important to involve the patient in handover. Where practicable, handovers should be conducted, in part, in the presence of the patient (e.g., at the bedside) or carer.
- Consider patient consent for handover to occur in the presence of carer/next of kin (NOK) or visitors.
- Patients should be provided with opportunities to seek clarification, ask questions and confirm information. Specifically, the outgoing nurse/midwife who is giving the handover should invite patients to comment or ask questions during the handover.
- Patient groups that may not participate in handover include those who are:
  - Asleep
  - Cognitively impaired
  - Comatose / sedated
  - In isolation
  - Have difficulty communicating in English (Refer to the [AKG Interpreter and translator guidelines](#))
  - Other conditions that preclude participation.

### 3.2 Correct identification and procedure matching

To ensure the patient receives the care intended for them:

- All patients will be identified using three core identifiers: full name, date of birth (DOB), and UMRN outlined in the [EMHS Patient Identification and Procedure Matching Policy](#)

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Confirm identity at every care point: admission, procedures, medication, handover, discharge.
- Use interpreters for patients (including their carers) who speak limited or no English Culturally and Linguistically Diverse (CALD) or hearing-impaired patients.

### 3.3 Consistent structure and content

Shift to shift handover will commence with a structured shift safety huddle (see appendices for examples)

- Clear, concise, and use easily understood words with minimal, accepted, abbreviations.
- Must use the iSoBAR format to guide the content and structure of the handover in a manner that suits the clinical context, as directed by the [HDWA Clinical Handover Policy](#).

To ensure that all clinical handovers use a consistent structure and content the following tool should be used as the basis of information exchange during a clinical handover. Table 1 outlines the basic principles of the iSoBAR framework, and [Appendix 1](#) outlines the framework in more detail.

Table 1: iSoBAR framework

|          |                                    |                                                                                                                                               |
|----------|------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------|
| <b>i</b> | Identification                     | Introduce or identify patient, self and team                                                                                                  |
| <b>S</b> | Situation                          | Provide current working diagnosis, specific clinical problems, concerns and critical laboratory results                                       |
| <b>O</b> | Observation                        | Check, update and discuss recent vital signs                                                                                                  |
| <b>B</b> | Background history                 | Update and discuss relevant medical and support information                                                                                   |
| <b>A</b> | Agree to a plan (actions)          | Outline plan for assessment, treatment and discharge                                                                                          |
| <b>R</b> | Responsibility and risk management | Confirm shared understanding; clarify tasks (read back critical information to check understanding), timing and responsibility is transferred |

### 3.4 Agreement on responsibilities and accountability

Handover must be understood by staff as an explicit transfer of clinical accountability and responsibility, not just information.

Roles, responsibilities, and accountabilities must be agreed to by all staff involved in the handover

### 3.5 Appropriate modality

Handover should be conducted face-to-face wherever possible. All inpatient handovers should include a verbal component wherever possible, such as: a current clinician responsible for the patient should speak directly to a receiving clinician prior to handover of responsibility or accountability.

In settings with non-continuous shift coverage, the [HDWA Clinical Handover Policy](#) recognises that a written only process is an adequate modality, however where possible, verbal clinical handover should also occur.

### 3.6 Appropriate environment

Environmental controls should be in place to limit non-critical interruptions to communication during handover. Consider confidentiality and privacy.

### 3.7 Supporting documentation

All handovers must be supported by current, appropriate documentation (clinical notes, test results etc.).

### 3.8 Patients of concern

Patient handover should be prioritised based on severity and clinical risk, as assessed by the treating clinician. Management of a deteriorating patient must be escalated as soon as deterioration in condition is detected as per the [AKG Recognition and Response to Acute Deterioration Policy](#)

## 4. Procedure

The following section provides multidisciplinary clinical staff with guidance about clinical handover, communicating for safety at the different touchpoints or transitions of patient care.

Transitions of care occur when all or part of a patient's care is transferred between healthcare providers, locations or different levels of care within the same location, as the patient's conditions and care needs change.

### 4.1 Admission to a health service

On admission to the health service, all patients should be asked to participate in:

- Patient identification and procedure matching
- Any informed consent and decision-making capacity
- Medication history and reconciliation
- Screening for risks and planning comprehensive care
- Cultural identification (e.g., Aboriginal or Torres Strait Islander status)

### 4.2 Shift to shift handovers

This procedure is intended as a guide to clinical staff when conducting a shift-to-shift changeover or team handover and may be amended dependent on clinical context

#### 4.2.1 General principles

1. Ensure that staff are allocated to maintain continuity of care during the handover period and to support the minimisation of interruptions during handover.
2. Prior to handover time and where appropriate, team members should inform patients (excluding sleeping, sedated or comatose patients) that the handover will begin shortly.
3. With the patient's permission, family members are allowed to stay at the patient's bedside during the handover. Other visitors should be asked to leave the room during the handover.
4. Update any clinical handover sheets and documentation in the clinical record to complement the verbal component of the clinical handover.
5. Commence the handover in a designated area at the designated time (see the Appendix 2 section for clinical area specific details).

6. Prioritise handover of patients in accordance with their severity and clinical risk, as determined by a treating clinician. All shift to shift changeovers/team handovers should identify all patients of concern and newly admitted patients.

#### 4.2.2 Safety scan process (general ward areas)

The Shift Coordinator will initiate the handover process by addressing the clinical context of the shift. This should take approximately 5 minutes and include:

- Notification about patients who might require significant levels of care or immediate attention, are clinically deteriorating or have the potential to deteriorate.
- Patients with an Advance Health Directive (AHD), Goals of Patient Care (GoPC stating if the patient is for active treatment or not for active treatment) and/or guardian status.
- Known admissions and discharges and other significant ward activity.
- Occupational health and safety issues (i.e., manual handling risks).
- Necessary updates (i.e., infection control risks, change in procedures).
- Include alerts such as recent codes or name suppression.

#### 4.2.3 'Huddle' handover

For shift-to-shift handovers in the ED, staff must proceed to the designated area at the commencement of their shift. Information shared during 'huddle' handover includes:

- Identification of team leads, shift coordinators and role allocation
- Note any patients formed Mental Health Act
- Identified behavioural, equipment and infrastructure hazards and/or risks in the department
- Any relevant ED news bulletins
- Clinical issues and unit updates i.e., influenza outbreaks.

#### 4.2.4 Bedside handover

The following procedure should be followed when conducting clinical handover at the bedside with the patient and/or their carer. This may occur as part of the shift-to-shift changeover/team handover process.

- As the outgoing staff member has already built a rapport with the patient, they should introduce the oncoming team members to the patient and family.
- During the bedside handover, oncoming staff should undertake a safety check of the patient's environment and equipment. Key items to consider are:
  - The patient's call bell is in reach
  - Dressings, drains, intravenous fluids and infusion pumps are secure and correct
  - The general tidiness of the area is conducive to safe mobility and ease of access
  - Any other checks that may be specific for that patient (e.g. use of bed rails, height of the bed, Invisa-beam® etc.).
- The oncoming staff member should review the bedside chart to identify additional safety concerns such as medications not signed for and changes in vital signs. This includes but is not limited to:
  - Patient Care Plan
  - Observation and Response Chart (including vital signs)

- Medication Charts (e.g., Hospital Medication Chart, Insulin Infusion Chart etc.)
- Fluid Balance Charts
- Any risk assessments (e.g., Falls Risk Assessment and Management Plan (FRAMP) or Skin Assessments and Wound Care Plan etc.)
- Any risks associated with hearing impairment, visual impairment or known physical disabilities
- Palliative Care Outcomes Collaboration Assessment
- Rounding Chart.

### **Patient confidentiality**

Patient confidentiality can be an issue if not adequately addressed. Several strategies can be used to ensure that patient confidentiality is maintained. For example:

- Sensitive information can be shared away from the bedside preferably discussed in a designated handover room
- Staff should lower their voices when sharing sensitive information
- Sensitive information may be recorded on the handover sheet.
- Sensitive information may include:
  - Blood tests of a diagnostic nature (e.g., HIV positive)
  - Communicable disease information (e.g., Hepatitis)
  - Mental health issues (e.g., suicidal, drug and alcohol abuse)
  - 'Not for Resuscitation' orders
  - Social and some family issues (e.g., conflicts, family and domestic violence).

#### **4.2.5 Non-continuous shift-to-shift handover considerations**

If participation at shift changeover and/or team handover is not practicable, at a minimum, a written clinical handover is to be documented in the iSOFT Clinical Manager system or the clinical record by a member of the treating team.

### **4.3 Allied Health considerations**

- It is recognised that for circumstances other than inter-facility patient transfer, not all patients require a formal allied health handover on all occasions. In some instances, informal handover that complies with the principles (described in the WA Health Clinical Handover Policy) may be sufficient.
- It is the responsibility of each individual clinician to determine whether appropriate care/services will be provided by the receiving clinician in the absence of formal handover, and if in doubt, the formal process must be completed.
- If participation at shift changeover and/or team handover is not practicable, at a minimum a written clinical handover is to be documented in the iSOFT Clinical Manager system by the allied health team

#### 4.4 Multidisciplinary handovers

The Shift Coordinator is to participate in the scheduled multidisciplinary handovers that occur on each ward/department

*Note:*

- Actions taken and agreed upon during clinical handover and communications regarding patient safety are to be documented in the patient's medical record.
- If the patient is not included or involved in the handover a justification for this should be documented in the patient's medical record (i.e., It is considered detrimental to the therapeutic relationship between clinical staff and the patient).

#### 4.5 Hospital nurse manager handovers

For handovers to and between Hospital Nurse Managers (HNM), the HNM is to be updated for all:

- Patients requiring one-on-one specialising.
- Pending admissions, discharges, and intra/inter hospital transfers.
- Patients with a current GoPC order.
- Bed capacity and any patient flow issues.
- Clinical and social situations of concern to patients and/or staff.
- Any facility, IT management issues.
- Any bypass events that have been escalated to SHOC.

#### 4.6 Change in care team or clinical setting within the same hospital

This procedure outlines the minimum requirements for the communication of information to support the coordination of a patient's movement between clinical settings and between teams.

- Intra-hospital transfers, where clinical responsibility is also transferred, should be verbally handed over (either face-to-face or telephone) by one of the providing clinicians to one of the receiving clinicians at the time, or prior to, transfer.
- In addition to a verbal component, a written summary of the handover is also to be completed and should arrive prior to, or with the patient, at the time of transfer.
- A safety huddle will be instigated prior to any intra-hospital transfer of an involuntary mental health consumer. The safety huddle will discuss identified risks and determine any appropriate strategies to ensure safe transfer to the destination ward. The Safety Huddle is to include security staff, Psychiatric Liaison Nurse (PLN), Duty Medical Officers (DMO and Psychiatric Registrar), and any other staff deemed necessary to attend.

#### 4.7 Escalation of care

To ensure safe, early and appropriate management of deteriorating patients and medical emergencies occurs across the service, any concern regarding a patient's clinical status (even in the presence of normal physiological observations) should be escalated as outlined in the [AKG Recognition and Response to Acute Deterioration Policy](#)

Communication with patients, family, carers and significant others should include:

- Education regarding patient, family and carer-initiated escalation of care processes – refer to the [Aishwarya's CARE Call Procedure](#).



- Discussion for patient centered information to inform assessment and interventions such as 'Top 5'.

#### 4.8 Transfer of care to another facility

All inter-hospital patient transfers must adhere to the principles outlined in the [Inter-Hospital Patient Transfer Policy \(EMHS: 38\)](#). This procedure outlines the minimum requirements for the communication of information to support the coordination of a patient's movement between hospitals.

- All patients for which responsibility is transferred inter-hospital should be verbally handed over (either face-to-face or telephone) by one of the providing clinicians to one of the receiving clinicians at the time, or prior to, transfer.
- In addition to a verbal component, a written summary of the handover is also to be completed and should arrive prior to, or with the patient, at the time of transfer.
- Medical governance must be obtained from the receiving site Consultant or delegate, prior to a patient transfer.
- Inter-facility and intra-department handovers must be supported by a transfer document or discharge summary, which should arrive prior to, or with the patient at time of transfer.

##### 4.8.1 Involuntary mental health considerations

A safety huddle will be instigated prior to any inter-hospital transfer of an involuntary mental health consumer. The safety huddle will discuss identified risks and determine any appropriate strategies to ensure safe transfer to the destination Hospital. The Safety Huddle is to include security staff, Psychiatric Liaison Nurse (PLN), Duty Medical Officer (DMO Psychiatric Registrar, if available) and any other staff deemed necessary to attend.

#### 4.9 Transfer for a test or appointment

Primary care providers to transfer accountability and responsibility on leaving and/or on receipt of patient transfer. Staff to notify shift coordinator/team leader of activity.

#### 4.10 Discharge

A discharge summary should be completed within 24 hours of a patient discharge and forwarded it to the relevant health care provider and to the patient/carer as per the relevant team discharge protocols.

All overnight admissions require a Discharge Summary. Surgical Same day admissions with completed Operation Record (AKMR95) do not need a discharge summary.

### 5. Compliance monitoring

Compliance against this procedure will be monitored by the Communicating for Safety Committee.

Compliance monitoring activities relevant to this procedure include:

- Quarterly auditing as per the AKG Audit and Reporting Framework.

### 6. Policy context

The following documents are required to give effect to this procedure:

## HDWA

- [Clinical Handover Policy MP0095/18](#)
- [Recognising and Responding to Acute Deterioration Policy MP0086/18](#)

## 7. Supporting information

The following documents support the implementation of this procedure:

- HDWA
  - [Clinical Handover Guideline](#)
  - [Clinical Handover Matrix](#)
  - [Mental Health Bed Access, Capacity and Escalation Policy](#)
- EMHS
  - [Inter-Hospital Patient transfer Policy \(EMHS:38\)](#)
  - [Patient Identification and Procedure Matching](#)
- AKG
  - [Aishwarya's CARE Call Procedure.](#)
  - [Interpreter and Translator Guidelines](#)
  - [Observations: Safe and Supportive Procedure](#)
  - [Recognition and Response to Acute Deterioration Policy](#)
  - [Identification of Patients in the Armadale Mental Health Service Procedure](#)
- Other
  - [Psychiatric Services Online Information System \(PSOLIS\) Service Events User Guide](#)

## 8. Relevant standards

- National Safety and Quality Health Service (NSQHS) Standards (second edition):
  - Standard 1 – Clinical Governance
  - Standard 2 – Partnering with Consumers
  - Standard 4 – Medication Safety
  - Standard 5 – Comprehensive Care
  - Standard 6 – Communicating for safety
  - Standard 8 – Recognising and Responding to Acute Deterioration
- Chief Psychiatrist's Standards for Clinical Care (2015)

## 9. References

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3. Desmedt, M., Ulenaers, D., Vanhove, A.-C., De Vos, M., Vanhaecht, K., & Hellings, J. (2020). Clinical handover and handoff in healthcare: A systematic review of systematic



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4. Ghosh, S., Ramamoorthy, L., & Pottakat, B. (2021). Impact of structured clinical handover protocol on communication and patient satisfaction. *Journal of Patient Experience*, 8, 1–6.  
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<https://blogs.bmj.com/ebn/2023/03/05/bedside-handovers-an-efficient-reliable-and-inclusive-nursing-handover-process/>
6. Philpin, S. (2006). "Handing over": Transmission of information between nurses in an intensive therapy unit. *Nursing in Critical Care*, 11(2), 86–93. <https://doi.org/10.1111/j.1362-1017.2006.00157.x>
7. Tataei, A., Rahimi, B., Lotfnezhad Afshar, H., Alinejad, V., Jafarizadeh, H., & Parizad, N. (2023). The effects of electronic nursing handover on patient safety in the general (non-COVID-19) and COVID-19 intensive care units: A quasi-experimental study. *BMC Health Services Research*, 23, Article 527. <https://doi.org/10.1186/s12913-023-09502-8>

## 10. Glossary

The following definitions are relevant to this procedure:

| Term                    | Definition                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
|-------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Clinician               | A person, registered under the Health Practitioner Regulation National Law (Western Australia) 2010, mainly involved in the area of clinical practice. That is the diagnosis, care and treatment, including recommended preventative action, to patients. Clinicians include allied health professionals, medical officers, midwives, and nurses.                                                                                                                                                                                                                                                                                                       |
| Intra-facility transfer | The transfer of responsibility of a patient within one health entity (under the same management), e.g. to/from operating theatre, departments or wards; inpatient to community mental health service; referral to a specialist; and escalation of a deteriorating patient. See also inter-facility handover.                                                                                                                                                                                                                                                                                                                                            |
| Inter-facility transfer | The move of an admitted patient between healthcare services where: they were admitted and/or assessed and/or received care and/or treatment at one service; and were admitted and/or received treatment and/or care at the second service. Services in WA include, but are not limited to: <ul style="list-style-type: none"> <li>• hospitals</li> <li>• community health services, e.g., mental health, child health, dental health</li> <li>• prisons</li> <li>• aged care facilities in home care services</li> <li>• supported accommodation</li> </ul> transport providers, such as St John Ambulance Service and the Royal Flying Doctors Service |

| Term               | Definition                                                                                                                                                                                                                                                                                               |
|--------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Patient of concern | A patient that a clinician is particularly concerned about, as defined by the treating clinician. This includes patients discharged from an Intensive Care Unit in previous 24 hours, any patient who has had a Medical Emergency Team call in last 24 hours and any other patients of clinical concern. |

## 11. Document contact

Enquiries relating to this procedure may be directed to:

Title: Director Allied Health and Ambulatory Care  
 Email: Danielle.kilmurray@health.wa.gov.au

## 12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version | Effective from | Effective to | Amendment(s)                                                               |
|---------|----------------|--------------|----------------------------------------------------------------------------|
| V3.1    | 09 12 2025     | 01 10 2028   | Minor Update to add Appendix 7 (Theatre Safety Huddle Proforma Principles) |

## 13. Authorisation

This procedure has been authorised and issued by Director Allied Health and Ambulatory Care:

|                           |                                                                 |
|---------------------------|-----------------------------------------------------------------|
| <b>Authorised by</b>      | Danielle Kilmurray – Director Allied Health and Ambulatory Care |
| <b>Authorisation date</b> | 08 12 2025                                                      |
| <b>Published date</b>     | 09 12 2025                                                      |

## Appendix 1: iSoBAR framework

| The iSoBAR Framework – Overarching Guidelines of Clinical Content |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|-------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Identify</b>                                                   | <ul style="list-style-type: none"> <li>Identify yourself and introduce relevant staff to the patient (where practicable).</li> <li>Identify the patient (at least three minimum identifiers must be used, i.e. Patient name, DOB and UMRN).</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                    |
| <b>Situation</b>                                                  | <ul style="list-style-type: none"> <li>Reason for handing over; diagnosis/presenting complaint / procedure / current status.</li> <li>Reason for referral.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
| <b>Observations</b>                                               | <ul style="list-style-type: none"> <li>Physiological observations (including observations and response chart scores (i.e. Adult Deterioration Detection System (ADDS) chart, Children's Early Warning Tool (EWS) score and the patient's monitoring plan (observations and their frequency)).</li> <li>Recent escalations of care.</li> <li>Pain score.</li> <li>Invasive devices.</li> <li>Medication efficacy / side effects.</li> </ul>                                                                                                                                                                                                                                |
| <b>Background</b>                                                 | <ul style="list-style-type: none"> <li>Pertinent information relevant to patient (medical, social, surgical, and mental health history).</li> <li>Medication allergies.</li> <li>Resuscitation Status / Advance Directives / Treatment limiting orders (For Medical Emergency Team (MET) activation OR Not For MET activation).</li> <li>Test results (comparison to previous), fluid balance, nutrition.</li> <li>Infection status.</li> <li>Risk; Falls, mobility, behavioral issues (i.e. aggression), skin integrity, VTE.</li> <li>Cultural and linguistic considerations.</li> <li>Medical and Allied Health updates.</li> <li>Mental Health Act status.</li> </ul> |
| <b>Agree a plan / Assessment</b>                                  | <ul style="list-style-type: none"> <li>Assessment of patient, actions taken, actions required, level of urgency.</li> <li>Determine and agree on an appropriate plan.</li> <li>Involve the patient / family or carer where practicable.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Recommendations / Read-back</b>                                | <ul style="list-style-type: none"> <li>Further considerations / anticipated treatments.</li> <li>Discharge goals / plans outlining risk(s) and required management.</li> <li>Clarification and confirmation of shared understanding.</li> <li>Document handover.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                               |

## Appendix 2: Clinical handover guidelines by type and department

Clinical handovers include, but are not limited to the clinical handovers outlined in the tables below:

| Where: All Departments (Inpatient and Outpatient Services) |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              |                                                                                                                               |                                                                                                                               |
|------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                  | When does clinical handover occur?                                                                                            | How should handover be delivered/documented?<br>Use iSoBAR framework     | Who should attend/be involved                                                                                                                                                                                                | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                               |
| Escalation of Care                                         | Immediately for any concern regarding a patient's clinical status (even in the presence of normal physiological observations) | Verbal (face to face/telephone) and document in patient's medical record | Person escalating care to provide information to relevant clinical staff member and inform/involve patient/carer/family where possible.<br>Escalation to Medical Review sticker to be filled out in areas where it is in use | I                                                                                                                             | Yourself (designation, date, time, location)<br>Patient (name, D.O.B, age) Documentation to be checked for correct patient ID |
|                                                            |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              | S                                                                                                                             | Reason for escalation/call – what, when and how severe                                                                        |
|                                                            |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              | O                                                                                                                             | Vital signs, recent escalations of care, invasive devices, pain score                                                         |
|                                                            |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              | B                                                                                                                             | Relevant history/new information, relevant clinical results, medications, allergies                                           |
|                                                            |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              | A                                                                                                                             | Your assessment of the patient, actions taken, actions required, level of urgency                                             |
|                                                            |                                                                                                                               |                                                                          |                                                                                                                                                                                                                              | R                                                                                                                             | Recommended intervention(s)<br>Clarify/confirm instructions/plan of care and estimated time of review document                |

| Where: All Departments                                                                                                                                 |                                                   |                                                                      |                                                                                                                                                                                      |                                                                                                                               |                                                                                                                                                                          |
|--------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------|----------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                                                                                                              | When does clinical handover occur?                | How should handover be delivered/documented?<br>Use iSoBAR framework | Who should attend/be involved                                                                                                                                                        | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                          |
| <b>Temporary or unexpected need to handover patient care</b><br>(e.g., meal breaks, transfer to medical imaging, necessary change in staff allocation) | As coordinated by Shift-Coordinator / Unit Leader | Verbal (face to face)                                                | Primary care providers to transfer accountability and responsibility on leaving and/or on receipt of patient transfer.<br>Staff to notify shift coordinator/team leader of activity. | <b>I</b>                                                                                                                      | Identify self and patient                                                                                                                                                |
|                                                                                                                                                        |                                                   |                                                                      |                                                                                                                                                                                      | <b>S</b>                                                                                                                      | Reason for presentation/admission/procedure<br>Current clinical status                                                                                                   |
|                                                                                                                                                        |                                                   |                                                                      |                                                                                                                                                                                      | <b>O</b>                                                                                                                      | Physiological concerns/issues, pain score, invasive devices, vital signs                                                                                                 |
|                                                                                                                                                        |                                                   |                                                                      |                                                                                                                                                                                      | <b>B</b>                                                                                                                      | Relevant history, consider risks (falls, aggression, self-harm, visiting restrictions, recent treatments/medications administered/withheld, infection status), allergies |
|                                                                                                                                                        |                                                   |                                                                      |                                                                                                                                                                                      | <b>A</b>                                                                                                                      | Outstanding actions                                                                                                                                                      |
|                                                                                                                                                        |                                                   |                                                                      |                                                                                                                                                                                      | <b>R</b>                                                                                                                      | Clarify and confirm for shared understanding                                                                                                                             |

| Where: <b>Operating Theatres</b> (Holding Bay or other designated area, including Day Procedure Unit Handovers) |                                                      |                                                                      |                                                                                                                                   |                                                                                                                               |                                                                                                                                                                                                                                                                                                                                                                                          |
|-----------------------------------------------------------------------------------------------------------------|------------------------------------------------------|----------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                                                                       | When does clinical handover occur?                   | How should handover be delivered/documented?<br>Use iSoBAR framework | Who should attend/be involved                                                                                                     | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                                                                                                                                                                                                          |
| <b>Pre-Operative</b><br><br>Handover of patient care to theatre staff from other ward/unit/Dept staff           | Pre-operative<br>Elective or Non-Elective Procedures | Verbal (face to face) and document in patient's medical record       | Where practicable, admitting, or primary nurse/midwife to escort patient and handover to theatre nurse or anaesthetic technician. | I                                                                                                                             | Self and patient (2 ID bands)                                                                                                                                                                                                                                                                                                                                                            |
|                                                                                                                 |                                                      |                                                                      |                                                                                                                                   | S                                                                                                                             | Check procedure with patient and documentation, ensuring procedural and anaesthetic consents are correct                                                                                                                                                                                                                                                                                 |
|                                                                                                                 |                                                      |                                                                      |                                                                                                                                   | O                                                                                                                             | Preoperative vital signs and invasive devices (i.e., IDC), pain score                                                                                                                                                                                                                                                                                                                    |
|                                                                                                                 |                                                      |                                                                      |                                                                                                                                   | B                                                                                                                             | Relevant medical and surgical history, allergies, pre-medications, address risks (infection status, mobility, hearing, visual, speech impairments), fasting status, blood results, tests undertaken. (ECG, pregnancy etc.) relevant history, consider risks (falls, aggression, self-harm, visiting restrictions, recent treatments/medications administered/withheld, infection status) |
|                                                                                                                 |                                                      |                                                                      |                                                                                                                                   | A                                                                                                                             | Correct procedure/operation site stated by patient and/or carer/guardian                                                                                                                                                                                                                                                                                                                 |
|                                                                                                                 |                                                      |                                                                      |                                                                                                                                   | R                                                                                                                             | Further recommendations/concerns. Clarify and confirm for shared understanding                                                                                                                                                                                                                                                                                                           |

| Where: Operating Theatres (Recovery)                                                              |                                    |                                                                      |                                                               |                                                                                                                               |                                                                                                                                                                                  |
|---------------------------------------------------------------------------------------------------|------------------------------------|----------------------------------------------------------------------|---------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                                                         | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework | Who should attend/be involved                                 | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                  |
| <b>Post-Operative</b><br><br>Handover of patient care from theatre staff to other ward/unit staff | As required post operatively       | Verbal (face to face) and document in patient's medical record       | Recovery nurse to receiving primary nurse (where practicable) | <b>I</b>                                                                                                                      | Self and patient (2 ID bands)                                                                                                                                                    |
|                                                                                                   |                                    |                                                                      |                                                               | <b>S</b>                                                                                                                      | Procedure including any complications and significant findings                                                                                                                   |
|                                                                                                   |                                    |                                                                      |                                                               | <b>O</b>                                                                                                                      | Pre- and post-operative vital signs, pain score and invasive devices, surgical incision sites                                                                                    |
|                                                                                                   |                                    |                                                                      |                                                               | <b>B</b>                                                                                                                      | Relevant medical/surgical history, medications including intra-operative medications, allergies, address risks (infection status, mobility, hearing, visual, speech impairments) |
|                                                                                                   |                                    |                                                                      |                                                               | <b>A</b>                                                                                                                      | Actions taken, post-op instructions/orders                                                                                                                                       |
|                                                                                                   |                                    |                                                                      |                                                               | <b>R</b>                                                                                                                      | Clarify and confirm for shared understanding                                                                                                                                     |



| Where: <b>ALL Departments</b>                                                                                    |                                    |                                                                      |                                                                                                                                  |                                                                                                                               |                                                                                                                                                                                                                                                |
|------------------------------------------------------------------------------------------------------------------|------------------------------------|----------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                                                                        | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework | Who should attend/be involved                                                                                                    | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                                                                |
| <b>Intra-hospital and Inter-hospital handovers</b><br><br><b>(Patient transfers to another ward or hospital)</b> | As required                        | Verbal (face to face) and document in patient's medical record       | Where practicable, primary nurse/midwife to receiving nurse/midwife (caregiver to caregiver). Involve patient where practicable. | <b>I</b>                                                                                                                      | Yourself, your role, the patient, location, patient details (3-point ID check)                                                                                                                                                                 |
|                                                                                                                  |                                    |                                                                      |                                                                                                                                  | <b>S</b>                                                                                                                      | Diagnosis, presenting complaint, procedure                                                                                                                                                                                                     |
|                                                                                                                  |                                    |                                                                      |                                                                                                                                  | <b>O</b>                                                                                                                      | Vital signs, ADDS/MADDS (PARROT score for children), escalations of care, invasive devices, pain score                                                                                                                                         |
|                                                                                                                  |                                    |                                                                      |                                                                                                                                  | <b>B</b>                                                                                                                      | Medical/surgical/social/mental health history/legal status, medications, allergies, resuscitation status/Advance health directives, test results, fluid balance, nutrition, falls, mobility, aggression, skin integrity, VTE, infection status |
|                                                                                                                  |                                    |                                                                      |                                                                                                                                  | <b>A</b>                                                                                                                      | What actions are required? How urgent? What is the plan of care/treatment?                                                                                                                                                                     |
|                                                                                                                  |                                    |                                                                      |                                                                                                                                  | <b>R</b>                                                                                                                      | Discharge goals/plans (consider risks). Clarify/confirm for shared understanding: who is doing what by when? Document.                                                                                                                         |

**Where: Medical Ward / Palliative Care**

| Type of clinical handover               | When does clinical handover occur?    | How should handover be delivered/documented?<br>Use iSoBAR framework                                                                                                                                                      | Who should attend/be involved                                                                                                                                                                                                   | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                                                                                                                                                                                              |
|-----------------------------------------|---------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Nursing Shift-to-shift Handover)</b> | Medical Ward<br><br>AM:<br>PM:<br>ND: | Attend Staff base for safety huddle then proceed to patient bedside (involving patient/carer where appropriate)<br><br>Clinical Handover documentation to be updated prior to each handover and during shift as required. | <b>Safety Huddle;</b> Both Shift Coordinators (Designated Leaders) and all incoming nursing staff<br><br><b>Bedside Handover;</b> outgoing primary nurse to oncoming allocated nurse involving patient/carer where practicable. | <b>I</b>                                                                                                                      | Identify self and patient (ID check), 3 unique identifiers, Patient's Doctor                                                                                                                                                                                                                                                                                                 |
|                                         |                                       |                                                                                                                                                                                                                           |                                                                                                                                                                                                                                 | <b>S</b>                                                                                                                      | Diagnosis/presenting complaint, date of admission including where patient was admitted from                                                                                                                                                                                                                                                                                  |
|                                         |                                       |                                                                                                                                                                                                                           |                                                                                                                                                                                                                                 | <b>O</b>                                                                                                                      | Vital signs, invasive devices, pain score                                                                                                                                                                                                                                                                                                                                    |
|                                         |                                       |                                                                                                                                                                                                                           |                                                                                                                                                                                                                                 | <b>B</b>                                                                                                                      | Medical/Surgical/Social history, AHD/ Resuscitation status/Goals of care (For Code Blue / MET/not for MET), behaviours (i.e., Aggression, cognitive impairment), hygiene, wound management plan, pressure injuries, mobility, alerts (i.e., infections status), Variable dose medications, allergies, Dietary requirements, Allied Health, and Medical updates/test results, |
|                                         |                                       |                                                                                                                                                                                                                           |                                                                                                                                                                                                                                 | <b>A</b>                                                                                                                      | Care/Treatment plan, Tests/Investigations, Actions taken and required                                                                                                                                                                                                                                                                                                        |
|                                         |                                       |                                                                                                                                                                                                                           |                                                                                                                                                                                                                                 | <b>R</b>                                                                                                                      | referrals Discharge plans/transport arrangements (consider risks). Clarify and confirm for shared understanding who is doing what by when?                                                                                                                                                                                                                                   |

| Where: <b>Surgical Wards</b>                          |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 |                                                                                                                               |                                                                                                                                                                                                   |
|-------------------------------------------------------|------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                             | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework                                                                                                                                                                       | Who should attend/be involved                                                                                                                                                                                                   | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                   |
| <b>Nursing</b><br><br><b>Shift-to-shift Handover)</b> | AM:<br>PM:<br>ND:                  | Attend <b>Staff base</b> for safety huddle <b>then</b> proceed to <b>patient bedside</b> (involving patient/carer where appropriate)<br>Clinical Handover documentation to be updated prior to each handover and during shift as required. | <b>Safety Huddle;</b> Both Shift Coordinators (Designated Leaders) and all incoming nursing staff<br><br><b>Bedside Handover;</b> outgoing primary nurse to oncoming allocated nurse involving patient/carer where practicable. | <b>I</b>                                                                                                                      | Identify self and patient (ID check), 3 unique identifiers, Patient's Doctor                                                                                                                      |
|                                                       |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 | <b>S</b>                                                                                                                      | Admission diagnosis/Procedure                                                                                                                                                                     |
|                                                       |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 | <b>O</b>                                                                                                                      | Vital signs, invasive devices, pain score, wound/surgical site observations                                                                                                                       |
|                                                       |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 | <b>B</b>                                                                                                                      | Medical/Surgical/Social history, wound management plan, mobility, alerts (i.e., infection status), medications, allergies, dietary requirements, Allied Health, and Medical updates/test results, |
|                                                       |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 | <b>A</b>                                                                                                                      | Care Plan Actions taken, Actions required, scheduled tests/investigations and referrals                                                                                                           |
|                                                       |                                    |                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                 | <b>R</b>                                                                                                                      | Discharge goals/plans/transport arrangements (consider risks). Clarify and confirm for shared understanding - who is doing what by when?                                                          |

Where: **Obstetrics and Neonates**

| Type of clinical handover                                         | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework                                                                                                                                                                           | Who should attend/be involved                                                                                                                                                                                                   | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                                                                               |
|-------------------------------------------------------------------|------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Midwifery / Nursing</b><br><br><b>Shift-to-shift Handover)</b> | AM:                                | Attend <b>Staff base</b> for safety huddle <b>then</b> proceed to <b>patient bedside</b> (involving patient/carer where appropriate)<br><br>Clinical Handover documentation to be updated prior to each handover and during shift as required. | <b>Safety Huddle;</b> Both Shift Coordinators (Designated Leaders) and all incoming nursing staff<br><br><b>Bedside Handover;</b> outgoing primary nurse to oncoming allocated nurse involving patient/carer where practicable. | <b>I</b>                                                                                                                      | Identify self , mother, and baby (ID check) 3 unique identifiers, G.P/Obstetrician and/or Consultant Obstetrician/Paediatrician                                                                                                                               |
|                                                                   | PM:                                |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                 | <b>S</b>                                                                                                                      | Reason for admission/current status, parity, gestation, previous caesarean section(s)                                                                                                                                                                         |
|                                                                   | ND:                                |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                 | <b>O</b>                                                                                                                      | Mother – vital signs/PV loss/Uterine Activity, pain score/ invasive devices. Neonatal observations                                                                                                                                                            |
|                                                                   |                                    |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                 | <b>B</b>                                                                                                                      | Medical/Surgical/Social/Pregnancy History, GBS Status, blood group, test results, alerts (i.e., infection status, aggression, visiting restrictions, CPFS: Child Protection and Family Support Services involvement), Social Work input, Physiotherapy needs, |
|                                                                   |                                    |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                 | <b>A</b>                                                                                                                      | Feeding/CTG Actions taken - neonatal Mother – agreed Care/Treatment Plan                                                                                                                                                                                      |
|                                                                   |                                    |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                 | <b>R</b>                                                                                                                      | Care plan. Actions required, scheduled tests/investigations/referrals.<br>Discharge goals/plans/transport arrangements (consider risks). Clarify and confirm for shared understanding - who is doing what by when?                                            |

**Where: Short Stay Unit (Emergency Department)**

| Type of clinical handover                             | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework                                                                 | Who should attend/be involved                                                                                                                                                                                                | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                       |
|-------------------------------------------------------|------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Nursing</b><br><br><b>Shift-to-shift Handover)</b> | AM:<br><br>PM:<br><br>ND:          | Attend <b>Staff base</b> for safety huddle <b>then</b> proceed to <b>patient bedside</b> (involving patient/carer where practicable) | <b>Safety Huddle;</b> Both Shift Coordinators (Designated Leaders) and all incoming nursing staff<br><br><b>Bedside Handover;</b> outgoing primary nurse to oncoming allocated nurse involving patient/carer where possible. | <b>I</b>                                                                                                                      | Identify Self and Patient, 3 unique identifiers                                                                                                                       |
|                                                       |                                    |                                                                                                                                      |                                                                                                                                                                                                                              | <b>S</b>                                                                                                                      | Reason for presentation/Diagnosis. Current clinical status.                                                                                                           |
|                                                       |                                    |                                                                                                                                      |                                                                                                                                                                                                                              | <b>O</b>                                                                                                                      | Vital signs, Invasive Devices, Pain Score                                                                                                                             |
|                                                       |                                    |                                                                                                                                      |                                                                                                                                                                                                                              | <b>B</b>                                                                                                                      | Patient history, allergies, reviews med charts, fluid balance charts, test results. Deficits/risks – falls risks, mobility aids, visual/hearing impairments,          |
|                                                       |                                    |                                                                                                                                      |                                                                                                                                                                                                                              | <b>A</b>                                                                                                                      | Care plan. Any tasks for completion e.g., ongoing ECG's, Investigations planned or pending results.<br><br>Actions taken                                              |
|                                                       |                                    |                                                                                                                                      |                                                                                                                                                                                                                              | <b>R</b>                                                                                                                      | Plan for discharge/transfer (consider risks)<br>Discharge goals/plans for admission.<br><br>Clarify and confirm for shared understanding – who is doing what by when? |

| Where: <b>Emergency Department</b>         |                                          |                                                                                                                                         |                                                                                                                               |                                                                                                                               |                                                                                                                                                             |
|--------------------------------------------|------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                  | When does clinical handover occur?       | How should handover be delivered/documented?<br>Use iSoBAR framework                                                                    | Who should attend/be involved                                                                                                 | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                             |
| <b>1. Shift Coordinator to Coordinator</b> | AM:<br>PM:<br>ND:<br><br>And as required | For verbal handover attend Shift Coordinator's desk                                                                                     | Shift Coordinators / Patient Flow-Coordinator and CNAM                                                                        | <b>I</b>                                                                                                                      | Identify patient and self                                                                                                                                   |
|                                            |                                          |                                                                                                                                         |                                                                                                                               | <b>S</b>                                                                                                                      | Reason for presentation. Current clinical status.                                                                                                           |
|                                            |                                          |                                                                                                                                         |                                                                                                                               | <b>O</b>                                                                                                                      | Vital signs, Invasive Devices, Pain Score                                                                                                                   |
| <b>2. Nursing shift-to-shift</b>           | AM:<br>PM:<br>ND:<br><br>And as required | Attend Flash Handover then proceed to patient bedside and refer to supporting documentation (involving patient/carer) where practicable | <b>Bedside Handover:</b><br>outgoing primary nurse to oncoming allocated nurse involving the patient/carer where practicable. | <b>B</b>                                                                                                                      | Patient history, allergies, reviews med charts, fluid balance charts, test results. Deficits/Risks – falls risks, mobility aids, visual/hearing impairments |
|                                            |                                          |                                                                                                                                         |                                                                                                                               | <b>A</b>                                                                                                                      | focused clinical assessment, Actions taken<br>Actions / tasks required e.g., ongoing ECG's, investigations planned or pending results.                      |
|                                            |                                          |                                                                                                                                         |                                                                                                                               | <b>R</b>                                                                                                                      | Discharge/admission/transfer plans (consider risks) Clarify and confirm for shared understanding – who is doing what by when?                               |

| Where: <b>Emergency Department for Mental Health Presentations</b> |                                    |                                                                      |                                                                                          |                                                                                                                               |                                                                                                                                                                                                                                                                                      |
|--------------------------------------------------------------------|------------------------------------|----------------------------------------------------------------------|------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Type of clinical handover                                          | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework | Who should attend/be involved                                                            | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                                                                                                                                                                                      |
| <b>Nursing shift-to-shift</b>                                      | AM:<br>PM:<br>ND:                  | Face to face                                                         | Nursing Shift Coordinator, incoming nursing staff and outgoing primary nurse to handover | <b>I</b>                                                                                                                      | Self and patient (3-point ID check)                                                                                                                                                                                                                                                  |
|                                                                    |                                    |                                                                      |                                                                                          | <b>S</b>                                                                                                                      | Diagnosis/reason for admission Legal status (e.g., prescribed forms).                                                                                                                                                                                                                |
|                                                                    |                                    |                                                                      |                                                                                          | <b>O</b>                                                                                                                      | Vital signs, Invasive Devices, Pain Score, Visual Observations/Agitation Score                                                                                                                                                                                                       |
|                                                                    |                                    |                                                                      |                                                                                          | <b>B</b>                                                                                                                      | Pertinent history including forensic, risk assessments (charts in use), resuscitation status, Medications/Allergies, risks (i.e., aggression, infection status), visiting restrictions, Level of observation and associated risk, escalations of care                                |
|                                                                    |                                    |                                                                      |                                                                                          | <b>A</b>                                                                                                                      | Actions required. Psychiatrist, Psych Liaison Nurse (PLN) and Allied Health Reviews if required. Mental Health escalations to Mental Health Group as required. Appointments/Tests, patient care plan, patient, carer and family involvement. Alcohol Withdrawal Scale where relevant |
|                                                                    |                                    |                                                                      |                                                                                          | <b>R</b>                                                                                                                      | Discharge / Transfer plans (consider risks), clarify and confirm for shared understanding – who is doing what by when?                                                                                                                                                               |

Where: **Paediatrics**

| Type of clinical handover              | When does clinical handover occur? | How should handover be delivered/documented?<br>Use iSoBAR framework                                                    | Who should attend/be involved                                                                                                                                                                                                           | What is the minimum data content for the clinical information that is to be handed over? (Include ALL applicable information) |                                                                                                                              |
|----------------------------------------|------------------------------------|-------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------|
| <b>Nursing shift-to-shift handover</b> | AM:<br><br>PM:<br><br>ND:          | <b>Staff base</b> for safety huddle then proceed to <b>patient bedside</b> (involving patient/carer) where appropriate. | <b>Safety Huddle:</b><br>Incoming and outgoing Shift Coordinator (Designated Leaders) and incoming nursing staff<br><br><b>Bedside Handover:</b><br>outgoing primary nurse to oncoming allocated nurse and patient/carer where possible | <b>I</b>                                                                                                                      | Identify self and patient (ID check), 3 unique identifiers, patient age and admitting Doctor's name. NOK/Carer               |
|                                        |                                    |                                                                                                                         |                                                                                                                                                                                                                                         | <b>S</b>                                                                                                                      | Diagnosis / Reason for presentation                                                                                          |
|                                        |                                    |                                                                                                                         |                                                                                                                                                                                                                                         | <b>O</b>                                                                                                                      | Vital Signs, PARROT, Invasive Devices, Age-Appropriate Pain Score                                                            |
|                                        |                                    |                                                                                                                         |                                                                                                                                                                                                                                         | <b>B</b>                                                                                                                      | Medical, surgical, social history. Visiting restrictions, CPFS involvement, medications/allergies, infection status.         |
|                                        |                                    |                                                                                                                         |                                                                                                                                                                                                                                         | <b>A</b>                                                                                                                      | Actions Taken, Actions required, Scheduled tests/investigations/referrals, Treatment/Care Plan                               |
|                                        |                                    |                                                                                                                         |                                                                                                                                                                                                                                         | <b>R</b>                                                                                                                      | Care plan. Discharge goals/plans (consider risks), Clarify and confirm for shared understanding – who is doing what by when? |



## Appendix 3: Shift Safety Huddles

### What is a Shift Safety Huddle

A Shift Safety Huddle is a short, stand-up meeting, 5 minutes or less, that is typically used at the start of each shift.

The Shift Safety Huddle should be a very brief, structured team check-in at the start of the shift, to improve situational awareness, plan for high acuity patients, level load work assignments and proactively address risk and improve safety for patients.

The Shift Safety Huddle gives the team a way to maintain a focus on safety. It can also provide a setting to look back at the previous day's activity on the ward/unit and evaluate previous performance and look for opportunities for improvement.

### Benefits of Shift Safety Huddle

The Safety Huddle creates a level of situational awareness on the ward/unit for actual or potential problems.

It alerts all caregivers to potential or actual clinical issues on the wards and ensures that all members of the ward team are aware of potential or actual issues for each team member.

Engage team in thinking and talking about checklist use, communication behaviours, and related safety work

Recognize issues in checklist use, communication behaviours, and related safety work to address by training, coaching, and revising tools and methods

Identify issues that need escalation and resolution beyond the team and supervisor

### Key Activities in a Shift Safety Huddle

Lead by the out-going shift coordinator and attended by the on-coming shift coordinator and all members of the on-coming shift.

Proforma to be followed and can be added to for specialty areas e.g- maternity/mental health.

On-coming shift coordinator:

- Introducing the whole team to each other.
- Assign patient allocations pre-Huddle
- Ensure all staff know where the safety equipment e.g. emergency trolley is located and who has specific roles in an emergency
- Reinforce how the team will work on the unit e.g. no pass call bell, and rounding
- Any specific messages for all staff

Out-going shift coordinator:

- Clinical issues and risks should be identified including any specific plans in place
  - Patients who are identified as Patients of Concern (PoC), have had a MET call or suffered significant clinical deterioration requiring a medical review in previous 24 hours
  - Patients who have had a fall in previous 24 hours
  - Patients who have had a clinical incident in previous 24 hours
  - Patients who are confused, delirious, demented?
  - Patients who have Goals of Care in place
- Aware of any security or behavioural issues on the ward with patients and their family
- There should be an opportunity for staff to validate/clarify any issues before the shift safety huddle is closed.
- Allied Health staff are invited to participate in the 1300hrs safety huddle.

## Appendix 4: Shift Safety Huddle Proforma – all areas

|                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|--------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Introduction</b>                              | <ul style="list-style-type: none"> <li>• Huddle run by previous shift coordinator, current shift coordinator introduces self</li> <li>• Welcome all staff and confirm all staff are present for shift</li> <li>• No of patients on ward</li> <li>• No of new admissions to ward in previous shift or not and admissions expected</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                          |
| <b>Patient Flow</b>                              | <ul style="list-style-type: none"> <li>• Current status of Hospital and or Emergency Department</li> <li>• Patients waiting for admission from ED, elective demand, interhospital transfers for ward</li> <li>• Patient's suitable for transit lounge and/or discharge</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| <b>Important Environmental Information</b>       | <ul style="list-style-type: none"> <li>• Location of emergency trolley</li> <li>• Fire exits</li> <li>• Emergency roles and responsibilities</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
| <b>Important Unit Information</b>                | <ul style="list-style-type: none"> <li>• Outline specific unit information e.g. no pass call bell, rounding must be done, report all variations and issues to Shift Coordinator</li> <li>• Staff opportunity to ask questions</li> <li>• SDN- Education needs for the shift</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| <b>Safety concerns &amp; Patients of Concern</b> | <ul style="list-style-type: none"> <li>• On the last shift were there safety issues, any other near miss events with equipment, medications, infection control issues</li> <li>• Patients who are identified as a PoC therefore have had a MET call or suffered significant clinical deterioration requiring a medical review in previous 24 hours</li> <li>• Psych review secondary to Code Black</li> <li>• Patients who have had a fall in previous 24 hours</li> <li>• Patients who are confused, delirious, demented?</li> <li>• Patients who have Goals of Care in place</li> <li>• Patients with additional precautions in place</li> <li>• Aware of any security or behavioural issues on the ward with patients and their family</li> </ul> |
| <b>Important Messages/ward Specific</b>          | <ul style="list-style-type: none"> <li>• Important unit/hospital messages e.g. policy/procedure changes, engineering messages</li> <li>• Allied Health present- issues or concerns</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
| <b>Conclusion</b>                                | <ul style="list-style-type: none"> <li>• Thank the team</li> <li>• Team heads off to undertake bedside handover with patient, family and previous shift team.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |

## Appendix 5: Shift safety huddle proforma – Emergency Department

|                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
|--------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Introduction</b>                              | <ul style="list-style-type: none"> <li>Huddle run by previous shift coordinator, current shift coordinator introduces self</li> <li>Welcome all staff and confirm all staff are present for shift</li> </ul>                                                                                                                                                                                                                                                                                                                                       |
| <b>Patient Flow</b>                              | <ul style="list-style-type: none"> <li>No. of patients in department</li> <li>No. admissions waiting for ward transfer</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                  |
| <b>Important Unit Information</b>                | <ul style="list-style-type: none"> <li>Outline specific unit information e.g. notices in ED News Bulletin</li> <li>Staff opportunity to ask questions</li> <li>SDN- Education needs for the shift- Resus roles</li> </ul>                                                                                                                                                                                                                                                                                                                          |
| <b>Safety concerns &amp; Patients of Concern</b> | <ul style="list-style-type: none"> <li>On the last shift were there safety issues, any other near miss events with equipment, medications, infection control issues</li> <li>RA1- patients of concern, formed, aggressive, Security specials</li> <li>Patients who are identified as a Patients of Concern</li> <li>Patients who have had a fall in previous 24 hours</li> <li>Patients who are confused, delirious, demented?</li> <li>Patients who have Goals of Care in place</li> <li>Patients with additional precautions in place</li> </ul> |
| <b>Important Messages</b>                        | <ul style="list-style-type: none"> <li>NUM/ANUM messages</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
| <b>Conclusion</b>                                | <ul style="list-style-type: none"> <li>Thank the team</li> <li>Team heads off to undertake bedside handover</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                             |

## Appendix 6: Shift Safety Huddle Proforma – Maternity

|                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
|--------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Introduction</b>                              | <ul style="list-style-type: none"> <li>Huddle run by previous shift coordinator; current shift coordinator introduces self (Afterhours Clinical Midwifery Specialist)</li> <li>Welcome all staff and confirm all staff are present for shift</li> <li>No of patients on ward, Antenatal Assessment unit, Labour Ward</li> <li>Admissions from overnight</li> </ul>                                                                                                                                                                                                            |
| <b>Patient Flow</b>                              | <ul style="list-style-type: none"> <li>Current status of Hospital and or Emergency Department</li> <li>Maternity Bypass across the Metro Area</li> <li>Women in AAU awaiting LW. Patients in LW to be transferred to postnatal ward</li> <li>Patients waiting for admission from ED, elective demand, interhospital transfers for ward</li> <li>How many confirmed discharges, how many potential</li> </ul>                                                                                                                                                                  |
| <b>Important Environmental Information</b>       | <ul style="list-style-type: none"> <li>Location of emergency trolley</li> <li>Fire exits</li> <li>Emergency roles and responsibilities</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                             |
| <b>Important Unit Information</b>                | <ul style="list-style-type: none"> <li>Outline specific unit information e.g. no pass call bell, postnatal visits coming to the ward, report all variations and issues to Shift Coordinator</li> <li>Staff opportunity to ask questions</li> <li>SDN- Education needs for the shift (which SDM is rostered on)</li> <li>Management queries who to seek out</li> </ul>                                                                                                                                                                                                         |
| <b>Safety concerns &amp; Patients of Concern</b> | <ul style="list-style-type: none"> <li>On the last shift were there safety issues, any other near miss events with equipment, medications, infection control issues</li> <li>Patients who are identified as a PoC therefore have had a MET call or suffered significant clinical deterioration requiring a medical review in previous 24 hours</li> <li>Medical obstetric risk (pph/pre-eclampsia/TPL/FDIU)</li> <li>Patients with additional precautions in place</li> <li>Aware of any security or behavioural issues on the ward with patients and their family</li> </ul> |
| <b>Important Messages/ward Specific</b>          | <ul style="list-style-type: none"> <li>Important unit/hospital messages e.g. policy/procedure changes, engineering messages</li> <li>Allied Health present- issues or concerns</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                     |
| <b>Conclusion</b>                                | <ul style="list-style-type: none"> <li>Thank the team</li> <li>Team heads off to undertake bedside handover with patient, family &amp; previous shift team.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                        |

## Appendix 7

| Theatre Safety Huddle Proforma Principles:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |                                                                                                                                                                                                                                                                                                                                              |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Aim:</b> Provide a brief, focussed and structured exchange of information about potential or existing safety risks which may impact patients, staff and any person accessing theatre services. <ul style="list-style-type: none"> <li>Held at the commencement of nursing shift for all oncoming nursing staff</li> <li>Shift coordinator to run shift safety huddle.</li> <li>Discussion points as per headings shown below.</li> <li>5-minute duration. Commence promptly</li> <li>Patient support service and AINs and other specials to attend all huddles when on shift.</li> <li>ISOBAR handover and patient introductions (ID Checks) to occur between team members (and patient) after completion of shift safety huddle.</li> </ul> |                                                                                                                                                                                                                                                                                                                                              |
| <b>Patient Flow Demands</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Current Hospital bed status and surgical bed availability<br>Planned elective lists<br>Pending known Emergency cases                                                                                                                                                                                                                         |
| <b>Important Environmental Information</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | Location of emergency trolley<br>Fire exits<br>Emergency roles and responsibilities Important unit/hospital messages e.g. policy/procedure changes, engineering messages                                                                                                                                                                     |
| <b>Important Unit Information</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | Outline specific unit information<br>Staff opportunity to ask questions<br>SDN Education needs for the shift (which SDN is rostered on)                                                                                                                                                                                                      |
| <b>Safety concerns &amp; Patients of Concern</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | Staff introductions to include new staff in the area<br>Safety issues from the previous 24 hours; Datix / Choir or any other near miss events with equipment, medications, infection control issues<br>Expected patients who have anaesthetic risks, medical alerts, infection prevention precautions, require interpreters or other issues. |
| <b>Conclusion</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | Thank the team                                                                                                                                                                                                                                                                                                                               |
| Date:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 1                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 2                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 3                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 4                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 5                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 6                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |                                                                                                                                                                                                                                                                                                                                              |
| Theatre 7                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Surgeon:<br>Anaesthetist:<br>Paediatrician:                                                                                                                                                                                                                                                                                                  |
| Duty Anaesthetist:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                              |
| Individual theatre MDT huddles are coordinated by the Anaesthetist / Surgeon, in the Theatre the prior to the start of each case, to provide effective communication between multidisciplinary teams and discuss key safety risks and concerns in an open environment to enhance patient outcomes.                                                                                                                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                              |

